



I'm Still Here Foundation 2026-2027 Innovation Program

FULL PROPOSAL

Congratulations on being selected as a finalist for an *I'm Still Here Innovation Grant*!

We invite you to submit this Full Proposal to provide a more detailed overview of your proposed project. Please complete all sections below to help us gain a comprehensive understanding of your approach, goals, and anticipated impact. You are welcome to copy information you provided in your original *Letter of Interest*, update and expand upon it.

Please note – the full proposal is the next step in the grant award process, *not all projects will be funded*.

Full Proposals are due by April 24, 2026. Award recipients will be notified by June 2, 2026.

Please send your completed proposal by email as a Word document or PDF to:
sharon@imstillhere.org

ORGANIZATION INFORMATION

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Year founded: 2011

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<https://www.arch.tamu.edu/impact/centers-institutes-outreach/chsd/> (Center for Health Systems & Design)

Check the box next to the organization type:

- ☐ A 501 (c)3 nonprofit
- ☐ A for profit organization with fiscal sponsor
- ☒ An individual with fiscal sponsor
- ☐ Government Agency

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PROJECT NARRATIVE

- **PROJECT TITLE**

Neighborhood Hunt: Mapping Memories, Walking Together

- **DETAILED PROJECT DESCRIPTION**

Walking is more than “exercise”! It is an everyday activity through which people stay physically active, reconnect with routines, and engage with the community. For people living with dementia (PlwD), walking is one of the most familiar and achievable activities in daily life.¹ Regular walking can offer meaningful functional and cognitive benefits. For example, a study found that walking at least 30 minutes four times per week was linked to a 20% increase in 6-minute walking test performance (i.e., a measure of exercise tolerance in PlwD), a 23% improvement in activities of daily living performance, and a 34% slower rate of cognitive decline among PlwD.² Furthermore, walking through meaningful neighborhood places can stimulate autobiographical memory, reinforce wayfinding skills, and create moments of enjoyment and accomplishment. The U.S. Surgeon General has urged the nation to make walking a priority, calling for walkable communities that support walking for people of all ages and abilities.³ However, many neighborhoods are not designed with the needs of PlwD in mind. Walkability barriers/challenges in the neighborhood can lead to reduced confidence in walking and a “shrinking world” experience among PlwD. In contrast, walkable neighborhoods with features such as familiar street corners and landmarks, comfortable benches, and local stores that act as memory anchors, can encourage healthy outdoor physical and social activities for PlwD.

Our project “**Neighborhood Hunt: Mapping Memories, Walking Together**” reframes walking as a structured neighborhood scavenger hunt game, aimed at transforming everyday walks into an enjoyable, memory-stimulating, and socially engaging experience embedded in familiar community settings. We will use a series of neighborhood-based, game-like walking activities to support PlwD and their care partners in the Bryan–College Station, Texas, in partnership with [Dementia Friends USA](#) through [Dementia-Friendly Bryan–College Station](#). We aim to:

1. **Make walking more fun, engaging, and supportive of independence for PlwD and their care partners.** We transform everyday walking into a motivating, physically active, socially connected, and cognitively engaging experience through mapping, clue-solving, and scavenger hunting. By practicing wayfinding along familiar routes, PlwD can build confidence in and strategies to support independent and routine walking. Later cycles may gradually increase challenge but with the introduction of co-designed temporary environmental cues.
2. **Identify neighborhood facilitators and barriers affecting walking for PlwD and their care partners in Bryan–College Station.** Through a series of activities, the project will document meaningful destinations, environmental supports and barriers, and in situ walking experiences. These findings will generate locally grounded evidence on how neighborhood conditions shape walking, comfort, safety, and engagement for PlwD and their care partners.
3. **Develop an evidence-based toolkit for future dementia-friendly community walking programs.** A practical program toolkit will be developed to disseminate materials (e.g., annotatable map templates, scavenger-hunt clue banks, and route-design guides), along with

¹ Lee, I.-M., & Buchner, D. M. (2008). The Importance of Walking to Public Health. *Medicine & Science in Sports & Exercise*, 40(7), S512. <https://doi.org/10.1249/MSS.0b013e31817c65d0>

² Venturelli, M., Scarsini, R., & Schena, F. (2011). Six-Month Walking Program Changes Cognitive and ADL Performance in Patients With Alzheimer. *American Journal of Alzheimer's Disease & Other Dementias*, 26(5), 381–388. <https://doi.org/10.1177/1533317511418956>

³ Office of the Surgeon General. (2015). Executive Summary from Step It Up!: Surgeon General's Call to Action. <https://www.hhs.gov/surgeongeneral/reports-and-publications/physical-activity-nutrition/walking-executive-summary/index.html>

implementation lessons from the Bryan–College Station project. These materials can guide future walking programs in other communities and help adapt the model to support more dementia-friendly and walkable environments.

Figure 1 presents an overview of the proposed project, including its timeline, “Map → Design → Hunt” (MDH) activities, measures, outcomes, and additional components.

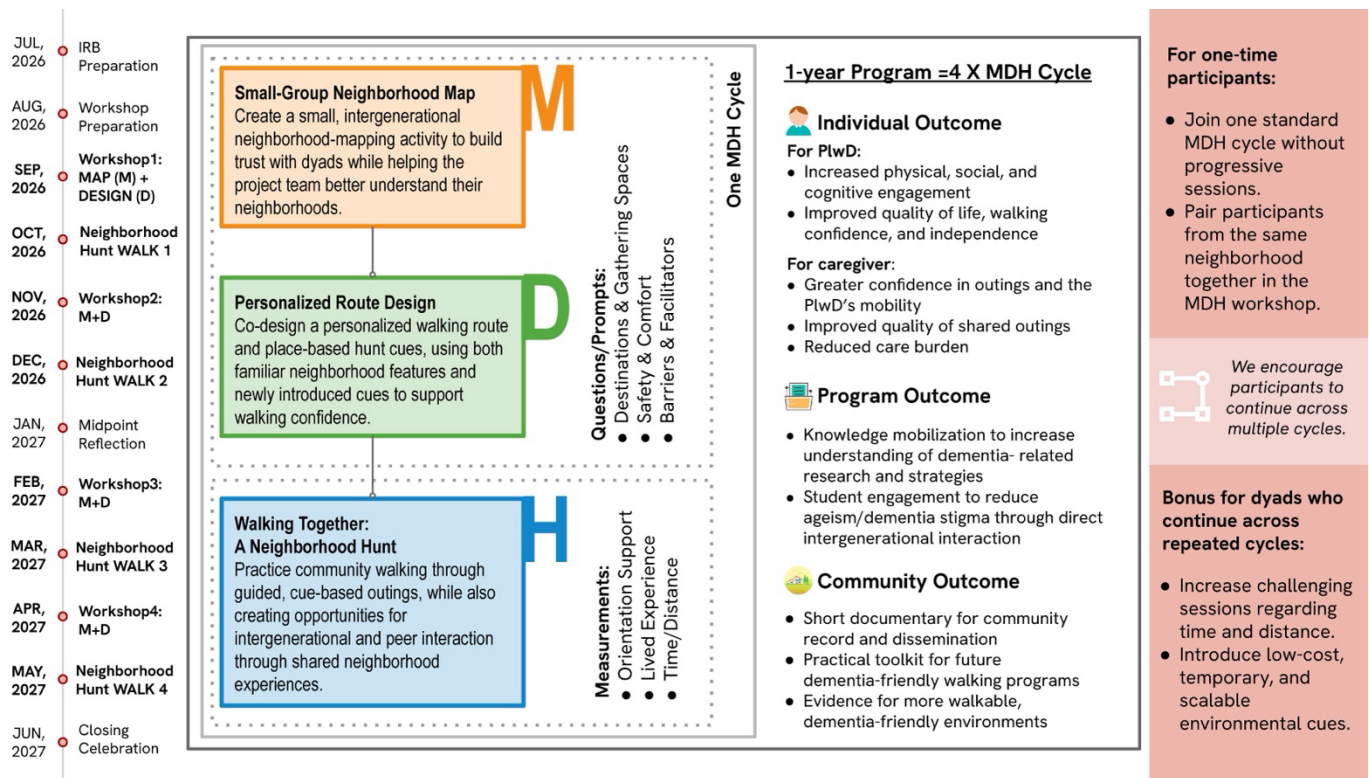


Figure 1. Overview of the “Neighborhood Hunt: Mapping Memories, Walking Together” Project

Detailed Project Design: Neighborhood Scavenger Hunt Walking Through “Map → Design → Hunt”

The project is structured around **repeated and evolving “Map → Design → Hunt” (MDH) cycles**, with each cycle designed for approximately five participating dyads of PlwD and their care partners (see the *TARGET POPULATION* and *PARTICIPANT RECRUITMENT* sections for additional details). In each cycle, the **MAP (M)** and **DESIGN (D)** steps are conducted together as workshop-based sessions in accessible community spaces (e.g., local public library meeting rooms), with student volunteers supporting intergenerational interactions. The Hunt (H) step is then carried out separately in each dyad’s neighborhood. After completing the first full MDH cycle, participants will be encouraged to continue in later cycles. For those who remain, subsequent sessions will become progressively more challenging, with gradual increases in walking distance and duration and the introduction of low-cost, temporary, and scalable environmental cues to enhance their interest and engagement.

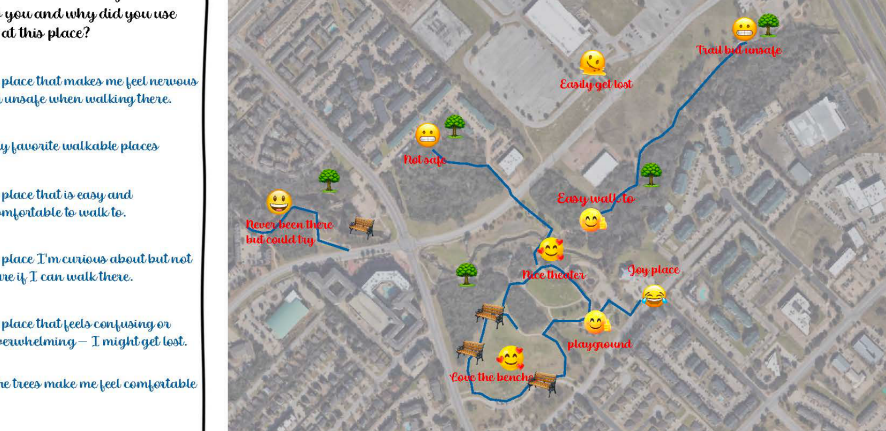
1. MAP (M) + DESIGN (D) WORKSHOP

1.1 Small-Group Neighborhood Mapping

We will hold multi-dyad (i.e., PlwD and their care partners) small-group sessions. Each dyad will work from a pre-prepared map of the area identified by the care partner as the place the PlwD most often visit for outdoor activity. Based on the map, PlwD will further define what they consider to be their own neighborhood. They are encouraged to mark meaningful destinations and manageable everyday touchpoints (e.g., favorite parks, familiar stores, shaded benches,

Emoji Key

	A place that makes me feel nervous or unsafe when walking there.
	My favorite walkable places
	A place that is easy and comfortable to walk to.
	A place I'm curious about but not sure if I can walk there.
	A place that feels confusing or overwhelming – I might get lost.
	The trees make me feel comfortable
	I like the benches there



The map shows a blue route line starting from the top left, passing through a park area with benches and trees, then through a residential area with a playground, and finally ending near a school. Annotations include:

- Top Left:** "Never been there but could try" (Thinking face emoji)
- Park Area:** "Not safe" (Nervous face emoji), "Love the benches" (Smiling face with halo emoji), "Place I'm curious about but not sure if I can walk there." (Thinking face emoji)
- Residential Area:** "Easy walk to" (Smiling face with sweat drop emoji), "Joy place" (Smiling face with hearts emoji), "playground" (Smiling face with halo emoji)
- Bottom Right:** "That's bad unsafe" (Nervous face emoji), "Easy get lost" (Smiling face with halo emoji)

Wolf Pen Creek District

During neighborhood mapping sessions, PlwD are guided through a set of place-based questions to help articulate their lived experience of the neighborhood. Rather than asking about the neighborhood in abstract terms, these questions anchor the conversation to specific locations on the map, prompting participants to identify where community life happens. Questions will cover three domains: (1) existing or potential walking destinations and gathering spaces, (2) safety and comfort along their familiar walking route, and (3) barriers to and facilitators for walking more. Below are some sample questions for each domain:

- 4

During the mapping activity, PlwD are encouraged to answer questions independently and lead the mapping process themselves. Care partners serve as a supporting person and offer additional clarification or assistance only when needed. Student volunteers will assist with facilitation and documentation. With participant permission, sessions may be audio- or video-recorded; meanwhile, student volunteers will complete field notes and brief reflection notes.

1.2 Personalized Route Design

After the mapping session, each dyad will work collaboratively with our team to co-design a personalized walking route for the later hunt activity. Locations identified during the mapping activity, along with the place-based activities and related perceptions or emotions described by participants (e.g., happiness, comfort, or stress), will be translated into scavenger-hunt clues.

Example clues include:

- Find the place where you like to sit and rest while watching people or cars go by.
- Go to a place that makes you feel happy because you often walk with someone you know.
- Find a place where you usually stop to look at flowers, trees, or other natural features.

2. NEIGHBORHOOD HUNT (H) WALK

Each dyad will complete a scavenger-hunt walk in their own neighborhood, accompanied by student volunteers. The hunt will use clues co-designed in the previous MAP (M) + DESIGN (D) Workshop. With participant permission, sessions may also be audio- or video-recorded; meanwhile, student volunteers will complete field notes and brief reflection notes. Student volunteers will document the Hunt process throughout the outing, with a focus on three areas:

- Orientation support needed by the PlwD
This will include the frequency, type, and source of assistance (e.g., identifying clues, confirming direction, making turning decisions), helping understand the level and nature of support required for neighborhood walking.
- Lived experience
At key decision-making points, participants will be asked to think aloud about what they are noticing, thinking, and feeling, and why they make particular wayfinding choices. Example questions adapted from prior community-based projects involving PlwD⁴ are as follows.
 - *What are you looking at while walking here?*
 - *What problems do you experience while you walk here? (Probes: Do the streets in this area pose problems? Is there anything here that you find confusing or overwhelming?)*
 - *What changes have you noticed recently in this area? (Probes: How do they affect your walks here?)*
 - *How do you decide which way to go? (Probes: At the corner where we turned, how did you decide which way to go? Do you see something on that street that helps you make the decision?)*
- Objective route information
Total walking duration, route distance, and time spent at major wayfinding or decision-making points will be recorded.

Each Hunt walk will include basic intergenerational interaction between dyads and student volunteers through shared walking, conversation, and route-based activities. In addition, because the project is focused within the Bryan-College Station community, dyads with

⁴ Seetharaman, K., Chaudhury, H., Hung, L., Phinney, A., Freeman, S., Groulx, M., Hemingway, D., Lanthier-Labonté, S., Randa, C., & Rossnagel, E. (2023). Protocol for A Mixed-Methods Study: Dementia-Inclusive Streets and Community Access, Participation, and Engagement (DemSCAPE). International Journal of Qualitative Methods, 22.

overlapping or nearby neighborhoods may be grouped in selected walks. This will create opportunities for peer social interaction while also strengthening community connections.

3. BONUS ENGAGEMENT ACTIVITIES

To support continued engagement, participants are encouraged to return for subsequent sessions through multiple strategies (see the *PARTICIPANT RECRUITMENT* sections for additional details). For returned dyads, each subsequent cycle will involve a gradual increase in challenge through longer walking distances and extended route duration if the care partner considers the change to be feasible. Concurrently, scavenger-hunt clues will become more varied and interactive. Drawing on the preceding mapping activities and students' interactions with participating dyads, later cycles may incorporate simple, low-cost, and temporary environmental cues along the walking route, co-designed by dyads and student volunteers. Examples of cues include custom-made universal signage, movable furniture, chalk markings on sidewalks, or permitted place-making installations. These elements will function as cues to help participants navigate and complete the more challenging neighborhood hunt walk.

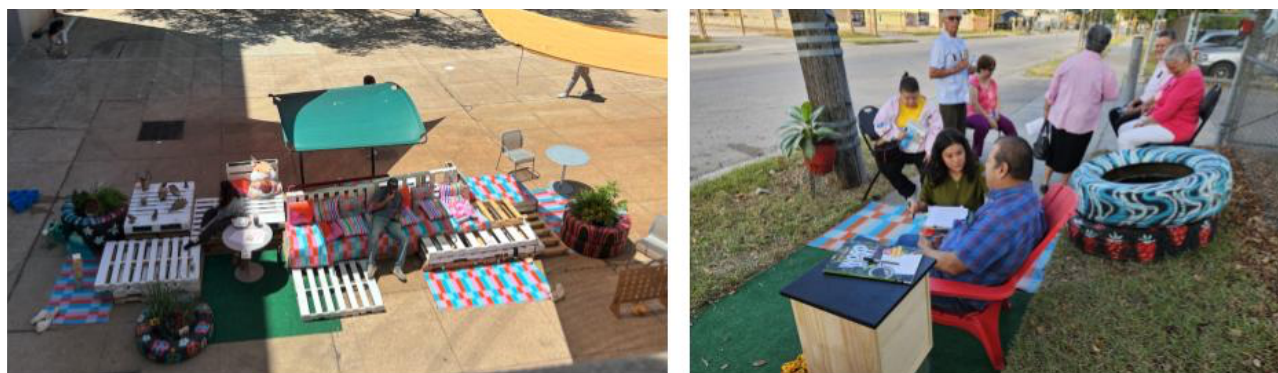


Figure 3. Previous Temporary Furniture Installation Projects by the DrAL Lab



Figure 4. Environmental Modification Examples

Figure 3 illustrates our prior experience with similar place-based interventions, making temporary furniture installations with students in public spaces on campus and creating a temporary neighborhood pop-up park for older adults in a Houston community. Figure 4 presents examples of feasible and engaging small-scale environmental modifications identified from online sources as references for our project (Sources and image credits are provided in the footnotes⁵).

4. LARGE-GROUP REFLECTION AND CLOSING ACTIVITIES

4.1 Mid-Point Reflection

During the winter period, MDH activities are paused due to seasonal weather conditions and

⁵ Image sources: the left image is from Tactical Urbanism: How Pop-Up Projects Ignite Community Development and Engagement; the middle image is from Dementia Friendly Neighbourhood | SUTD Social Urban Lab (SOULab); and the right image is from 7 Best Art Therapy Exercises for Dementia (Scholistico).

holiday scheduling. We will hold a mid-point reflection session instead. It will be a focus group with participating dyads, student volunteers, and the project team. The session will be used to gather feedback on earlier activities, review participants' and students' experiences, and make any needed adjustments to the implementation of the spring phase (3rd and 4th MDH cycles).

4.2 Final Reflection and Closing Celebration

The project will conclude with a closing celebration with participating dyads, student volunteers, and the project team. Beyond summarizing the program and reflecting its activities, the event will include shared viewing of photo and video documentation from earlier activities. Attending participants will receive certificates of completion and additional recognition awards such as Best Mapper, Best Designer, and Best Hunter. This will help acknowledge participant accomplishments and reinforce the value of continued participation in community-based activities and regular physical activity.

- **INNOVATION**

1. **Neighborhood as the venue built for real-life carryover.** This project uses the neighborhood, where PlwD spent most of their time, as the project setting without requiring additional facilities or specialized resources. By anchoring activities in familiar streets, parks, and everyday destinations of PlwD, we make it easy for PlwD and their care partners to integrate this into their daily routine, enabling long-term benefits.
2. **One simple walk with three-in-one benefits.** The proposed walking activity is structured to provide multiple health benefits simultaneously: 1) physical activity (neighborhood walking), 2) social interaction (small-group and intergenerational interactions), and 3) cognitive stimulation (mapping, clue-solving, and wayfinding). This program makes walking enjoyable, motivating, and accessible, while contributing to the health and wellbeing of PlwD and their care partners.
3. **Participants as contributors to dementia-friendly communities.** The project positions PlwD and care partners not only as participants, but also as contributors to their community. Their choices, comments, and observations become vital inputs into understanding lived neighborhood experience, which is essential for designing better evidence-based guides to walkable dementia-friendly neighborhoods. The project will also translate these insights into shareable products. Therefore, lessons learned in BCS can be adapted in other similar communities and sustained after the project ends.
4. **Embedded Knowledge Mobilization.** As part of the community-based activities, including the MAP (M) and DESIGN (D) Workshops, mid-point reflection, and closing celebration, we will invite our lab's faculty and student members to give brief public talks and share handouts or other materials with participants. These talks will introduce relevant research on age- and dementia-friendly environments and create opportunities for participants to ask questions. This component supports knowledge mobilization by translating research into accessible, community-facing formats.

- **TARGET POPULATION**

1. **For PlwD**, the inclusion criteria will be:
 - Older adults living in community settings rather than institutional care settings
 - Diagnosis of mild cognitive impairment (MCI) or mild-to-moderate stage dementia
 - No major difficulty with walking that would prevent participation in walking activities
 - Availability of a care partner to participate together as a dyad

This project focuses on community-dwelling PlwD, as approximately 85% of PlwD in the United

States, nearly 5.9 million people, live in community settings rather than in care facilities.^{6,7} For PlwD, the neighborhood contributes meaningfully to their social health and well-being and is the primary setting in which every day social participation and routine activities take place.⁸

2. **For PlwD's care partners**, the inclusion criteria will be:

- Providing direct caregiving support to PlwD who meet the above-mentioned eligibility criteria in most weeks
- Familiar with the care recipient's daily routine and the surrounding community and environment

Care partners will participate throughout all project stages to support participant comfort, safety, confidence, and engagement. In the MAP (M) and DESIGN (D) workshops, care partners will provide complementary information and serve a supportive role. When recall is difficult, care partners' long-term familiarity with the participants' routines and neighborhood experiences can help support the process and reduce discomfort or frustration. In the Hunt (H) process, care partners will also help ensure participant safety, including reducing fall risk and responding to other concerns.

We will seek to recruit a diverse group of participants and will make reasonable efforts to promote balance in gender, race, ethnicity, and other relevant demographic characteristics within the constraints of project scope and community availability.

● PARTICIPANT RECRUITMENT

We will recruit participants through, but not limited to, the following community-based channels in the Bryan-College Station area.

1. **Local aging-service organizations:** We will recruit through our local community partner [Dementia Friendly Bryan-College Station](#), for which our lab faculty member, *Dr. Marcia Ory*, serves on the Board as Community Services Sector Representative. We will also recruit from organizations such as the Area Agency on Aging of the Brazos Valley and Alzheimer's Association Houston & Southeast Texas Chapter through our community partner.
2. **Care partner support channels:** We have collected local dementia care partner support group resources. To support trust-building, we plan to establish contact in advance and, when appropriate, gradually engage with relevant group activities, and then distribute project information to reach care partners and dyads who may be eligible.
3. **Prior community-based research relationships:** We have built long-term community relationships through earlier research projects in the Bryan-College Station area. These include a sub-project funded by the U.S. National Institutes of Health (NIHR01CA197761), *Dementia-Friendly Communities to Promote Active Living in Persons with Alzheimer's disease and related dementias (AD/ADRD)*, and a project funded by the College of Architecture at Texas A&M University, *DESIGN FOR HEALTH: Age-friendly Communities to Promote Healthy Aging in Place*.
4. **Broader community outreach:** Additional recruitment may occur through local senior centers and other community-serving organizations in the Bryan-College Station area. For example, we may draw on existing contacts from walking-related programs affiliated with Texas A&M University, such as *Walk Across Texas*, to help reach community members who may already be interested in walking and physical activity.

⁶ National Health and Aging Trends Study. (2019). Community-Dwelling Older Adults with Dementia and Their Care partners: Key Indicators from the National Health and Aging Trends Study. <https://aspe.hhs.gov/sites/default/files/private/pdf/260371/DemChartbook.pdf>.

⁷ Centers for Disease Control and Prevention (CDC), 2024. Alzheimer's Disease and Dementia. <https://www.cdc.gov/alzheimers-dementia/about/index.html>. (accessed 12 May 2025).

⁸ Clark, A., Campbell, S., Keady, J., Kullberg, A., Manji, K., Rummery, K., & Ward, R. (2020). Neighbourhoods as relational places for people living with dementia. *Social Science & Medicine*, 252, 11292

To encourage participation among PlwD, we will:

1. Include the project's core activities on a recurring basis, with one MAP (M)+DESIGN (D) WORKSHOP and one NEIGHBORHOOD HUNT (H) WALK to be completed every two months, allowing trust and familiarity to be built gradually over time. After each completed cycle, participating dyads will be included in a small gift card lottery to recognize their time and engagement and to encourage continued participation in later cycles.
2. Provide periodic public talks by faculty members and student researchers from our lab to introduce existing age- and dementia-related research in an accessible way. PlwD and their care partners will gain exposure to current research developments, helping them build greater awareness and understanding of aging- and dementia-related topics.
3. Include a brief social time in each workshop to encourage informal interaction among PlwD, care partners, and invited undergraduate and graduate student volunteers from Texas A&M University. It will support continued participation and strengthens connections between community members and the university.

• GOALS

As noted above in the *DETAILED PROJECT DESCRIPTION* section, our short-term goal is to support more engaging and independent neighborhood walking, identify environmental facilitators and barriers for local walkable neighborhood development, and produce practical resources for future dementia-friendly walking programs. Our long-term goal is to help PlwD and their care partners maintain their walking habits after the project ends.

- For PlwD, walking offers physical, cognitive, and everyday quality-of-life benefits, especially when it takes place in meaningful neighborhood settings that support familiarity, engagement, and social connection. The project is designed to strengthen their self-efficacy and confidence in navigating neighborhood routes and enhance overall health and quality of life through meaningful engagement.
- For care partners, this project responds to a well-documented area of need, as 12 million Americans provide unpaid care for a family member or friend with dementia (valued at more than \$413 billion), and one of the most commonly reported stressors is difficulty getting help or taking a break.^{9, 10} As PlwD's confidence and independence in walking improve, the project may provide respite opportunities and contribute to reduced care burden and stress.

Success for this project will be reflected in whether these intended changes and outputs can be demonstrated through both measurable indicators and narrative documentation (see the *PROJECTED OUTCOMES* section for details). It is not only completing the planned workshops, but seeing participants meaningfully engage in the MDH process, choose to continue participating in multiple cycles, and maintain neighborhood walking habits beyond the project period. The project will be successful if it increases comfort and confidence in neighborhood walking for PlwD, improves shared outing experiences for care partners, and produces practical outputs that inform more walkable, dementia-friendly environments and future community walking programs.

• PROJECTED OUTCOMES

We will document outcomes at three levels:

1. At the individual level

1.1 For PlwD: We expect participation in repeated MDH cycles to encourage greater physical activity

⁹ Alzheimer's Association. (2025). 2025 Alzheimer's Disease Facts and Figures. <https://www.alz.org/getmedia/ef8f48f9-ad36-48ea-87f9-b74034635c1e/alzheimers-facts-and-figures.pdf>

¹⁰ Alzheimer's Association. (2024). New Alzheimer's Association Report Reveals Top Stressors for Care partners and Lack of Care Navigation Support and Resources. <https://www.alz.org/news/2024/new-alzheimers-association-report-reveals-top-stressors-care-partners>.

through neighborhood walking, increase social engagement through workshop participation and intergenerational interaction, and provide cognitive stimulation through mapping, co-design, clue-solving, and route-based decision-making. Together, these experiences are intended to enhance overall quality of life and strengthen comfort, confidence, and capacity for more independent walking in familiar community settings.

- 1.2 For care partners: We expect care partners to improve confidence during outings, support a more positive perception of the PlwD's independent range and mobility, and improve the quality of shared outings. By reducing some of the stress and uncertainty associated with community walking, the project may also modestly reduce perceived care burden.

In addition to the interview transcripts, observational notes, and objective route data collected during project activities (see the *DETAILED PROJECT DESCRIPTION* section for details), we will also conduct a brief survey or interviewer-administered check-in with care partners as reliable proxy^{11,12} before participation and again at the end of the project. This survey will assess areas such as PlwD's physical activity, social engagement, cognitive status, and perceived independence, as well as care partners' shared outing experiences, confidence during outings, and care burden.

2. At the program level

This project is expected to generate a structured but flexible model for dementia-friendly neighborhood walking programs. Through the MDH cycles, the program will support knowledge mobilization by translating research on age-friendly and dementia-friendly environments into accessible public talks, handouts, and other community-facing materials. It is also expected to increase participating dyads' understanding of dementia-related research and practical strategies.

By providing students with meaningful experience in participant support, documentation, and reflective learning, the project will strengthen student engagement in community-based aging- and dementia-related work and may help reduce both ageism and dementia-related stigma by creating direct, positive, and sustained intergenerational interaction. This outcome will be explored through brief interviews with student volunteers after they complete their volunteer involvement.

3. At the community level

The project is expected to produce outputs that extend beyond the immediate participants and support broader community awareness and future adaptation.

- 3.1 A short documentary will be created using project-generated photos, audio, and video materials collected with participant permission. It will serve as both a meaningful community record and a dissemination product, preserving participant stories and showing what this type of initiative can look like in practice. More broadly, the short documentary is expected to increase awareness of age- and dementia-friendly neighborhood environments in the Bryan-College Station area and strengthen connections among local residents and community partners.

- 3.2 A practical "how-to" toolkit will be developed to support future adaptation of the program in other communities. The toolkit will document the overall MDH model, recommended workshop structure, examples of mapping activities and hunt clues, strategies for developing place-based cues and interventions, and practical lessons learned from implementation. It will also include guidance on recruitment, participant support, student involvement, and documentation. Therefore, other communities can understand both the process and the materials needed to

¹¹ Arons, A. M., Krabbe, P. F., Schölzel-Dorenbos, C. J., Van der Wilt, G. J., & Rikkert, M. G. O. (2013). Quality of life in dementia: a study on proxy bias. *BMC Medical Research Methodology*, 13, 1-8.

¹² Neumann, P. J., Araki, S. S., & Gutterman, E. M. (2000). The use of proxy respondents in studies of older adults: lessons, challenges, and opportunities. *Journal of the American Geriatrics Society*, 48(12), 1646-1654.

adapt the program for their own local settings.

3.3 Local evidence will be generated to inform more walkable, dementia-friendly environments. All project activities will be documented, and lab researchers will further analyze participant-generated materials, walking records, reflections, and observational notes to produce more systematic evidence on neighborhood facilitators, barriers, and lived experiences. These findings can serve as a reference for future dementia-friendly community development in Bryan–College Station, including both new planning efforts and improvements to existing settings.

● REPLICABILITY

This project is designed to be adaptable by other communities. The repeated MDH cycles combined with reflection activities, and community-based dissemination is practical and flexible. Most activities rely on accessible community spaces, familiar neighborhood settings, basic materials, and collaboration with local partners rather than highly specialized facilities or personnel.

Replication may be especially feasible in communities with dementia support groups, caregiver networks, or related local partners willing to assist with outreach and implementation. This project is also structured to support replication beyond the grant period through two shareable products: 1) The short documentary (see the *PROJECTED OUTCOMES* for details) can provide other communities a clear and concrete sense of what the activities look like in practice and help make the project's format, atmosphere, and community impact more visible and understandable; and 2) The “how-to” toolkit with the project structure, major activities, and implementation lessons should enable other communities and organizations to adapt the model to their own settings, thereby supporting future replication and long-term sustainability.

Together, these products are intended to make the project both visible and usable beyond the Bryan–College Station area. The implementation strategies and lessons learned from this project can be adapted by other communities to develop dementia-friendly walking programs, community engagement efforts, and intervention designs that fit their own local contexts and needs.

● IMPLEMENTATION SCHEDULE

Time	Activities	Notes
Jun 2026	● Institutional Review Board (IRB) preparation*	Project setup will begin immediately after notification.
Jul–Aug 2026	● IRB review period ● Partner coordination ● Workshop preparation	Because this project involves a vulnerable population, the university's IRB review and approval process may take longer than usual, and sufficient time has therefore been built into the project schedule before participant-facing activities begin.
Sep–Dec 2026	● Workshop1: MAP(M) +Design(D) ● Neighborhood Hunt WALK 1 ● Workshop2: M+D ● Neighborhood Hunt WALK 2	Sep–Dec is generally a favorable period for outdoor walking in Texas with typical average temperature ranging from 88°F to 66°F and little risk of fall-season ice on walking surfaces.
Dec 2026 –Jan 2027	Mid-point reflection	<u>With participants:</u> Reflect on experiences from the first two MDH cycles to adjust plans for later phase. <u>Within lab:</u> Prepare for preliminary review materials.
Feb–May 2027	● Workshop3: M+D ● Neighborhood Hunt WALK 3 ● Workshop4: M+D ● Neighborhood Hunt WALK 4	Feb–May generally provides a suitable spring window for outdoor walking in Texas with typical average temperature ranging from 63°F to 84°F, and icy ground conditions are not normally a concern.

Time	Activities	Notes
Jun 2027	• Final reflection • Closing celebration	<u>With participants:</u> • Conduct a closing reflection with participants. • Screen the project's short documentary. • Present certificates for completion and small recognition awards. <u>Within lab:</u> • Organize/synthesize materials and records. • Prepare final products and project outputs.

Note: * Because this project will generate research-related data involving human participants, review by the university's IRB is required. The IRB is responsible for reviewing research involving human subjects to help ensure participant rights, safety, privacy, and ethical treatment are protected.

• CHALLENGES

The main challenges we foresee are participant recruitment and the university's IRB process.

1. Recruitment may take additional time because the project involves PlwD and their care partners as dyads and depends on strong community partnerships and trust-building. As noted in the *PARTICIPANT RECRUITMENT* section, we will leverage our established local networks, including local dementia-friendly organizations and other community-serving partners. Importantly, our previous community-based projects have established trusted relationships. They have also strengthened the local visibility of our work, which will in turn support recruitment and participant engagement for this project.
2. As the project involves a vulnerable population, university IRB review and approval may take longer than usual. To address this, we have built sufficient time into the project schedule before participant-facing activities begin. In addition, our team is currently undergoing IRB review for another project that directly involves PlwD, which allows us to draw on recent experience and incorporate guidance from IRB personnel into the preparation of this project's materials

ALIGNMENT WITH ISHF MISSION

• DEMENTIA ADAPTED ENGAGEMENT

Based on prior research and relevant guidance, this project adopts a dementia-adapted engagement approach that emphasizes secure and supportive settings, ample time for participation, repeated interaction, flexible semi-structured activities, and a positive sense of contribution.^{13,14,15,16,17} This approach is reflected in the following ways:

1. **Recurring and progressively adapted engagement:** The project is structured as recurring MDH cycles so that PlwD can gradually become familiar with the project, the team, and the activities over time. This repeated format allows participants to engage step by step, build rapport and confidence, and share at a comfortable pace.
2. **Care partner continuity and co-designed support:** Care partners remain involved throughout

¹³ Cridland, E. K., Phillipson, L., Brennan-Horley, C., & Swaffer, K. (2016). Reflections and recommendations for conducting in-depth interviews with people with dementia. *Qualitative Health Research*, 26(13), 1774–1786.

¹⁴ Nygård, L. (2006). How can we get access to the experiences of people with dementia? Suggestions and reflections. *Scandinavian Journal of Occupational Therapy*, 13(2), 101–112.

¹⁵ Thoft, D. S., Ward, A., & Youell, J. (2021). Journey of ethics—Conducting collaborative research with people with dementia. *Dementia*, 20(3), 1005–1024.

¹⁶ Hellström, I., Nolan, M., Nordenfelt, L., & Lundh, U. (2007). Ethical and methodological issues in interviewing persons with dementia. *Nursing Ethics*, 14(5), 608–619.

¹⁷ Lloyd, V., Gatherer, A., & Kalsy, S. (2006). Conducting qualitative interview research with people with expressive language difficulties. *Qualitative Health Research*, 16(10), 1386–1404

the process, supporting recall and communication when needed and adding an additional layer of safety and trust. Routes are co-designed with each dyad, paced according to participant comfort, and gradually adjusted in complexity to maintain confidence while reducing risks related to disorientation or unsafe navigation.

3. **Familiar community settings:** Activities are grounded in accessible community public spaces, participants' neighborhoods and everyday destinations. These recognizable environmental cues may support orientation and make the walking activities meaningful and relevant to daily life.
4. **Socially meaningful participation:** Walking is framed as a shared and socially meaningful activity rather than an isolated task. Each phase of the MDH process creates opportunities for interaction among PlwD, care partners, and student volunteers, helping participants remain socially visible, emotionally connected, and actively involved in community life.
5. **Accessible communication and presentation:** Questions, prompts, and materials will be adjusted to the abilities of PlwD and presented in clear, supportive, and easy-to-understand ways. Both wording and visual presentation will be simplified to reduce cognitive burden and support participation.

- **INCLUSIVITY**

This project is expected to help reduce stigma through four closely related approaches.

1. It challenges the non-person stereotype by positioning PlwD as active participants and contributors in the local community. In each phase of the MDH cycle, they are invited to identify meaningful places, respond to questions, define what counts as "their" neighborhood, and co-design personalized walking routes. The project is intended to encourage PlwD to contribute based on their own knowledge and experiences.
2. It addresses marginalization by creating regular opportunities for PlwD's visible participation in community life. Rather than separating them into a clinical setting, the project brings them into accessible public spaces together with care partners, student volunteers, and other community members, helping rebuild the social ties and showing that they still belong in public life.
3. It works against internalized self-stigma by creating repeated experiences of competence, enjoyment, and accomplishment. By grounding activities in familiar settings and involving PlwD in the design of goals and routes, the project may help them gradually build confidence and restore a sense of self-worth through ongoing engagement. This process is further strengthened by positive acknowledgment of their participation and achievement at the end of activities, such as small rewards and certificates.
4. It engages in intergenerational interaction through direct, positive interaction between students and PlwD, which may help reduce both ageism and dementia-related stigma. Through shared activities and conversation, students can come to see PlwD as capable community members with valuable perspectives, thereby supporting broader community inclusion.

- **ALIGNMENT WITH THE I'M STILL HERE PRINCIPLES**

Our project aligns closely with the I'm Still Here Principles by creating community-based opportunities for PlwD to engage, connect, and contribute through the repeated MDH cycles.

1. STILL HERE: Through MDH activities, participants are engaged as full persons. They identify meaningful places, help design routes, respond to prompts, solve clues, and share neighborhood experiences based on their own knowledge and preferences.
2. AUTHENTIC ENGAGEMENT: The project turns everyday walking into an engaging activity by using real community settings and meaningful place-based tasks. Mapping familiar streets, designing actual walking routes, and completing hunt walks in participants' own neighborhoods make the activities immediate, relevant, and grounded in daily life.

3. **PURPOSEFUL:** Each activity is tied to meaningful goals such as identifying places that matter, co-designing practical walking routes, and building confidence for neighborhood outings. A sense of purpose will be fostered by enabling participants to directly contribute to route design, community understanding, and final dissemination products.
4. **SELF EXPRESSION:** Participants are encouraged to express themselves through mapping, route discussion, think-aloud comments during walks, and reflection activities. These formats allow PlwD to communicate their feelings, preferences, and lived experiences in multiple ways.
5. **COMPETENCE:** The project is designed to create achievable success. Routes are co-designed and paced to participant comfort. Clue-solving is tailored to familiar places. Repeated cycles allow PlwD to build confidence, demonstrate existing knowledge, and strengthen walking and wayfinding skills over time.
6. **CHOICE:** Choice is built into the project at every stage, as their preferences remain central throughout the process. Participants are encouraged to make their own decisions about where to go, what route to take, and how the activities should be paced.
7. **SHARING ENGAGEMENT:** Care partners are included throughout the whole process. They support recall and communication during mapping and co-design sessions, help provide reassurance and safety during walks, and share the outing experience with PlwD.
8. **COMMUNITY:** The project is intentionally community-based. Activities take place in accessible local venues and participants' own neighborhoods. It helps make dementia engagement more visible within community life. The final short documentary will also serve as a meaningful community record, preserving local stories and experiences, while helping share the value of dementia-friendly community engagement more broadly.
9. **REACH OUT:** The project will make every effort to recruit local community-dwelling PlwD and their care partners through multiple local channels. In addition, the project's final "how-to" toolkit is intended to make the model useful beyond the immediate local context by supporting adaptation in other settings and communities.
10. **SPREAD THE WORD:** This project will share its impact through workshops, public talks, community handouts, the short documentary, and the toolkit with public. It will also recognize ISHF's support in these materials and relevant dissemination outputs, including conference presentations, academic publications, and professional newsletter articles, as appropriate.

COLLABORATION

• ACKNOWLEDGEMENT

We will help spread the word about ISHF's mission by acknowledging ISHF's support in project materials, public-facing events, and dissemination products whenever appropriate. This includes:

1. Mentioning ISHF in workshop materials, public talks, community handouts, and the project's final documentary and toolkit.
2. Sharing the project process and outcomes with participants, community partners, students, and broader local audiences to increase awareness of ISHF's commitment to community-centered, health-promoting initiatives.
3. Recognizing ISHF as a sponsor in project-related conference presentations, academic publications, professional newsletter articles, and other relevant dissemination outputs.

• COMMUNITY INVOLVEMENT / AWARENESS

This project is embedded in local community life. This project is rooted in Bryan-College Station and carried out through local relationships. It works with dementia-friendly and aging-related community networks while locating activities in accessible public venues and participants' own neighborhoods.

In this way, the project keeps dementia engagement visible within ordinary community life rather than separating it into a clinical setting.

The project also promotes reciprocal community awareness and education. Community involvement in this project is not one-directional. For student volunteers and other project members, direct and sustained interaction with PlwD may increase awareness and help reduce ageism and dementia-related stigma. For PlwD and their care partners, the project offers accessible opportunities to learn about and engage with research on age- and dementia-friendly environments in ways that are relevant to their daily lives.

The project extends community value beyond the project itself. The outputs are designed to carry community involvement forward after the activities end. The short documentary broadens awareness of dementia-friendly community engagement. The practical “how-to” toolkit will translate the project’s structure, activities, and lessons into a resource that other communities can adapt. In addition, project materials and findings may contribute to reports, publications, and presentations that inform policymakers, practitioners, and other professionals working to promote dementia-friendly communities.

RESOURCES NEEDED

• ANTICIPATED STAFFING NEEDS

1. Faculty/Lab Manager Project Lead (in-kind)
 - Provide overall project leadership and administrative coordination.
 - Coordinate with community partners.
 - Supervise the activity planning and delivery.
 - Prepare and submit IRB materials.
 - Deliver public talks on the lab’s existing dementia- and aging-related research.
2. Graduate/Undergraduate Student Facilitators (scholarship support)
 - Support activity delivery, participant engagement, logistics, and documentation.
 - Assist with handouts, records, audio/visual materials, and dissemination products.
 - Assist with data organization and analysis for academic dissemination.
3. Student Volunteers (volunteer; gift card encouragement)
 - Support participant interaction and activity implementation across the MDH process.
 - Assist with permitted audio/video recording, field notes, and reflection documentation.
4. Community Partner Liaisons (in-kind)
 - Support recruitment, outreach, and coordination with participants and care partners.

• VOLUNTEER RECRUITMENT

Student volunteers will be recruited primarily from Texas A&M University through multiple channels:

1. Lab recruitment: Undergraduate and graduate students affiliated with our lab will be invited.
2. Faculty teaching channels: Lab faculty members may recruit student volunteers by sharing opportunities in their courses and by encouraging participation through course-based activities, such as class projects, extra credit opportunities when appropriate.
3. Student organizations: We may recruit through Texas A&M student organizations, including Aggies Serving the Aging Population (ASAP), to broaden outreach and engagement.

• COMMUNITY PARTNERS

As noted above in the *PARTICIPANT RECRUITMENT* section, Dementia Friendly Bryan–College Station will serve as our main community partner. Our lab faculty member, Dr. Marcia Ory, serves on

the Board as the Community Services Sector Representative and is well-positioned to support coordination and community connection for this project. A letter of support from Dr. Ory is provided at the end of this document.

- **SPACE**

MAP(M)+DESIGN(D) Workshops will be held primarily in publicly accessible spaces, with local public library meeting rooms as a preferred venue. The library makes meeting rooms available at no charge to eligible community groups in Brazos County for educational and informational activities. The spaces offer convenient public access, parking, and basic meeting amenities. Because our lab is affiliated with Texas A&M University, workshops may also be held in publicly accessible campus spaces, including classrooms and other meeting areas when needed. The arrangements are a practical advantage of the project's community-based design, which allows us to make use of local public and partner resources while also benefiting from the university-based setting of our lab. Access to both community and campus spaces helps support low-cost, accessible workshop delivery, and no regular rental or admission fees are anticipated. Neighborhood Hunt (H) Walk activities will take place in participants' own neighborhoods. Using familiar streets, parks, and everyday destinations reduces logistical burden, supports participant comfort, and makes the project more sustainable and repeatable over time.

- **PROJECT SPECIFIC NEEDS**

The project does not require specialized facilities or equipment. It only requires basic supplies to support the workshop and Hunt activities, such as:

- Printed Materials: neighborhood maps, worksheets, public talk handouts, session agendas, instructions, and other clue-related paper materials.
- MDH Supplies: markers, pens, stickers, labeling dots, poster paper, cue boards, instruction cards, and small hunt gifts.
- Environmental Intervention Materials: low-cost materials used to introduce temporary place-based cues and supportive environmental elements (e.g., custom-made signage, chalk markings, portable markers, movable seating, and planters).

- **FEASIBILITY**

The project is designed to be feasible within one year and within the grant budget because it has a clear timeline, a focused set of activities, and a practical use of existing resources:

1. **Time feasibility:** The project timeline is structured to remain manageable within one year. Sufficient time has been built into the summer schedule for the university's IRB review and approval process before participant-facing activities begin, and the main activities are limited to four MDH cycles across fall and spring, with a winter reflection period and a wrap-up in June.
2. **Space and material feasibility:** The project minimizes costs by relying on accessible community spaces, such as public venues and participants' own neighborhoods, rather than rented or specialized facilities. Required materials are basic and relatively low-cost, including printing, markers, stickers, boards, and other simple activity supplies. In addition, some materials from lab previous projects may be reused when appropriate, helping reduce new material costs and support efficient
3. **Personnel feasibility:** The project will be carried out primarily by lab members, university students, and community partners through existing roles, in-kind support, and volunteer contributions, which helps keep staffing needs realistic and cost-efficient.

These factors help ensure that the project can be completed within both the grant period and the

available budget.

- **BUDGET**

- 1. Participant Incentives (Total cost: \$1,200)**

- 1.1 Participant Gift Card Drawing (\$400): Gift cards will be awarded to participants at the end of each MDH cycle through a small lottery with multiple award levels. A total of \$100 will be allocated in each cycle for the approximately five participating dyads to encourage continued participation and recognize engagement. For each cycle, gift cards will be distributed at multiple levels: one \$50 gift card, one \$20 gift card, and three \$10 gift cards.
- 1.2 Student Volunteer Gift Card (\$400): Gift cards will be provided (\$20 per student) at the end of each MDH cycle to recognize student volunteers' time, effort, and support in workshop facilitation and participant engagement. We anticipate at least five volunteers per cycle.
- 1.3 Additional Recognition / Contingency (\$400): To cover extra unanticipated participant-support costs that may arise during the project.

- 2. Workshop printing and materials (Total cost: \$500):**

To cover printing and paper-based materials for four workshops and one mid-point reflection session, including: neighborhood maps for mapping and route design; participant worksheets and activity sheets; reflection sheets for participants and student volunteers; public talk handouts; session agendas; instructions; and clue-related materials for workshop activities. A breakdown of the estimated costs is provided below.

- Printed neighborhood maps: \$150
- Participant worksheets and activity sheets: \$75
- Reflection sheets for participants and student volunteers: \$75
- Public talk handouts: \$100
- Session agendas, instructions, and clue-related paper materials: \$100

- 3. “Map → Design → Hunt” supplies (Total cost: \$1,000):**

Funds will support the core materials needed to carry out the MDH activities across repeated cycles. These supplies include markers, pens, stickers, labeling dots, poster paper, and foam boards for mapping and design activities, as well as cue boards and instruction cards to support the walking sessions. Low-cost hunt gifts will be used to acknowledge successful clue-solving and destination finding, while supporting motivation and positive engagement. A breakdown of the estimated costs is provided below.

- Markers and pens: \$150
- Stickers and labeling dots: \$150
- Poster paper and foam boards: \$250
- Cue boards and instruction cards: \$250
- Small hunt gifts: \$100

- 4. Intervention supplies (Total cost: \$2,000):**

This category will support the development of low-cost, temporary, and scalable environmental cues for participants who continue into repeated MDH cycles. Examples include custom-made signage, cue markers, portable visual prompts, and small environmental support (e.g., temporary seating, sidewalk chalk markings, planters). This category supports the project's core intervention strategy of pairing increased walking challenges with enhanced environmental cueing.

- Materials for student-made signage: \$300
- Materials for temporary chalk wayfinding markings and visual cues: \$200
- Portable cue markers and route anchors: \$200

- Small movable furniture (e.g., temporary seating, planters): \$600
- Materials for other low-cost, temporary environmental cues: \$700

To supplement this budget, we will reuse materials from previous projects, including temporary seating elements, small planters, and other low-cost environmental installations. Materials introduced in each cycle will also be recycled and reused in subsequent cycles whenever feasible. In addition, based on our prior experience with similar activities, local grocery stores and university event support units may be able to contribute some of the needed materials in kind.

5. Participant support (Total cost: \$1,000):

This category will provide reimbursement for local participants' transportation expenses, as needed, for attending community-based project activities. Each participating dyad may receive one transportation reimbursement of up to \$20 per event. Eligible activities may include Map+Design workshop sessions, the mid-point reflection session, and the closing celebration. These funds are intended to reduce practical barriers to participation and support continued engagement.

6. Refreshments (Total cost: \$1,000):

Funds will support healthy snacks and drinks for older adults, care partners, and student volunteers across 10 project activities (\$100 per activity). Providing refreshments will help create a welcoming and supportive workshop environment and facilitate informal social interaction during sessions.

7. Student Facilitator Scholarship support (Total cost: \$5,000):

Student scholarships are budgeted to support student facilitators for the project who will manage all proposed activities. We anticipate supporting approximately five student facilitators (\$1000/student), who will receive thorough training prior to their engagement in this project.

8. Closing celebration materials (Total cost: \$300)

8.1 Certificates of Completion (\$120): Cost for papers, printing, and framing is budgeted for the certificates, which will be used to provide formal recognition of participation for both project participants (i.e., PlwD and care partners) and student volunteers at the closing celebration.

8.2 Small Recognition Awards (\$180): Three engraved award plaques (\$60 for each) are budgeted to recognize the most engaged participant in each of the three events (i.e., mapping, co-design, and hunting).

Please see our budget form next page and letter of support at the end of this document.

BUDGET FORM

	General project expense	Cost for workshop 1 <i>MAP& DESIGN</i>	Cost for neighborhood hunt 1	Cost for workshop 2 <i>MAP& DESIGN</i>	Cost for neighborhood hunt 2	Cost for Mid-point reflection	Cost for workshop 3 <i>MAP& DESIGN</i>	Cost for neighborhood hunt 3	Cost for workshop 4 <i>MAP& DESIGN</i>	Cost for neighborhood hunt 4	Cost for closing celebration
Participant Incentives (PlwD and care partners)	\$1200, including a \$400 contingency fund		\$100		\$100			\$100		\$100	
Participant Incentives (student volunteer)			\$100		\$100			\$100		\$100	
Workshop printing & materials	\$500	\$100		\$100		\$100	\$100		\$100		
“Map → Design → Hunt” supplies	\$1000	\$100	\$100	\$100	\$100	\$100	\$100	\$100	\$100	\$100	\$100
Intervention materials	\$2000				\$1000			\$500		\$500	
Transportation	\$1000	\$100	\$100	\$100	\$100	\$100	\$100	\$100	\$100	\$100	\$100
Refreshments	\$1000	\$100	\$100	\$100	\$100	\$100	\$100	\$100	\$100	\$100	\$100
Student facilitator scholarships	\$5000	One-time scholarship support (\$1000/student) for student facilitators, to be awarded at the end of the project.									
Closing celebration materials	\$300										\$300



Dear I'm Still Here Foundation Grant Review Committee,

I am pleased to provide this letter of support for the project, *Neighborhood Hunt: Mapping Memories, Walking Together*, led by the Design Research for Active Living Lab at Texas A&M University. In my role as **Community Services Sector Representative on the Board of Dementia Friendly Bryan-College Station**, I am very supportive of this project and its goals to promote safe, meaningful, and more independent neighborhood walking for people living with dementia (PlwD) and their caregivers in the Bryan-College Station community.

Dementia Friendly Bryan-College Station is committed to fostering an inclusive and supportive community for individuals living with dementia and their care partners through education, advocacy, collaboration, and community engagement. The proposed Map → Design → Hunt program aligns strongly with this mission by creating practical, community-based opportunities for social connection, physical activity, and neighborhood engagement. The project's focus on familiar community settings, caregiver involvement, and accessible public programming reflects the values of dignity, belonging, and inclusion that are central to our organization's work.

Dementia Friendly Bryan-College Station is also well positioned to support community outreach for this project through our established resource and referral network. Our public-facing resource network includes connections to organizations such as the Brazos Valley Area Agency on Aging, the Aging and Disability Resource Center, the Texas Area Agency on Aging Caregiver Support Program, and the Alzheimer's Association Houston & Southeast Texas Chapter, along with other caregiver support and education resources. These connections can help increase awareness of the project and extend outreach to local people living with dementia and their caregivers.

Given my long-standing work in aging and public health and my involvement in dementia-friendly community initiatives, I see this project as highly consistent with current efforts to support aging in place, caregiver well-being, and inclusive community participation. I am confident that this collaboration will make a meaningful contribution to Bryan-College Station and will help build stronger connections among researchers, community organizations, caregivers, and people living with dementia.

I am pleased to support this proposal and look forward to continued collaboration and community engagement in support of this project.

Sincerely,



Marcia G. Ory, PhD, MPH

Regents and Distinguished Professor

Community Services Sector Representative, Board of Dementia Friendly Bryan-College Station

Director, Healthy Across the Generations